

Paediatric Emergency Department- Sedation Checklist

(To be used in conjunction with Procedural Sedation PED Guideline)

Affix Sticker

Consultant:.....

Weight: (Kg)

Sedationist:	Grade	Procedure:
Assistant:	Grade	
Procedurist:	Grade	
Supervisor:	Grade	Date: / /

Contraindications:

Parent Refusal	Y / N	Proposed procedure in mouth/pharynx	Y / N
Age <12 months	Y / N	Active respiratory infection	Y / N
Proposed procedure likely to take longer than 20 minutes	Y / N	Severe asthma	Y / N
No guarantee of adequate supervision post discharge	Y / N	Severe neurodevelopmental disorder	Y / N
Past adverse reaction to Ketamine	Y / N	Abnormal or unstable airway	Y / N

Fasting State

Food:

Time:.....

Clear Fluids:

Time:.....

Sedation Appropriate

Y / N

(Likely to be painful, short duration & alternative methods considered)

Risks Explained:

Apnoea	Y / N	Pain	Y / N
Hypotension	Y / N	Awareness	Y / N
Allergy	Y / N	Other:	

Procedural Checks:

Monitoring attached & functional	Y / N
Airway Rescue present & available (NPA/OPA/BVM/Bougie/LMA/ETT/O ₂)	Y / N
Procedurist present & ready	Y / N
Procedural Equipment available	Y / N

Allergies:

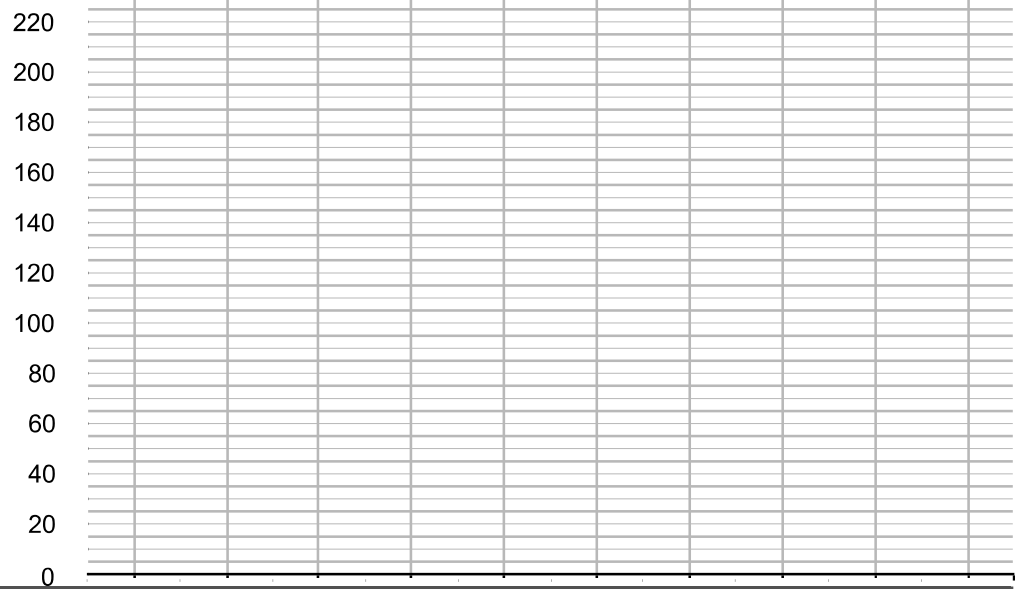
Signature:

Name & Grade:.....

Paediatric Emergency Department- Sedation Checklist

(To be used in conjunction with Procedural Sedation PED Guideline)

Monitoring (All must be present)		Technique + Adjuncts		IV Access	Position
Pulse Oximetry		Intravenous		Cannula 1:	
ECG		Entanox / Gas + Air		Cannula 2:	
End Tidal CO2		Nerve Block			
Non-invasive BP					

[illegible]

Time									
Spo2									
ETCO2									
Sensation Score									
Location <->									

Key:

BP: $\vee$
 $\wedge$

Pulse: ●

Resp Rate: X

University of Michigan Sedation Scale

0. Awake & Alert
1. Minimally sedated: tired sleepy, appropriate response to verbal conversation or sound
2. Moderately Sedated: Somnolent/sleeping, easily aroused with light tactile stimulation or verbal command
3. Deeply sedated: Deep sleep,aroused only with significant physical stimulation
4. Unroutable

Paediatric Emergency Department- Sedation Checklist

(To be used in conjunction with Procedural Sedation PED Guideline)

Serious Adverse Event (Requires DATIX completion)	Tick	Minor Adverse Event	Tick
Airway Problems		Agitation	
Cardiac arrest		Hypotension	
Hypoxia		Nausea/Vomiting	
Apnoea		Respiratory Depression	
Cardiovascular collapse		Allergy without anaphylaxis	
Anaphylaxis		Other	

Interventions	Tick	Notes
None		
Airway manouvres		
Bag valve mask ventilation		
ETT / LMA / NPA / OPA		
Admit for adverse event		
Other (please specify in notes box)		

Post Sedation Pre Discharge Checklist	Tick
Child's conscious level returned to baseline	
Vital signs within normal limits for child	
Resolution of nystagmus	
Able to walk unassisted	
No respiratory compromise	
Absence of significant pain and discomfort	
Adequate supervision can be assured	
Information leaflet for discharge provided	
Name Signature	

Sedating your child with ketamine to reduce distress during a procedure

Children's Emergency Department

Last reviewed: June 2024

Next review: June 2027

Information for Patients, Parents & Carers

Leaflet number: 1175 Version: 2

Why is my child being sedated?

Your child needs a medical treatment (for example stitches, treatment of a broken bone) which may be painful or distressing for them. So that this treatment can be carried out more easily, with less distress and pain for your child, the medical team wish to use a drug called ketamine to sedate them.

Sedation is a process of using medicine to make them sleepy and relaxed. It is normal that they may not remember the procedure at all, or remember only small amounts.

Ketamine is commonly used in hospitals for sedation of children. Ketamine is given via an injection into a vein. Your child will be cared for by a senior doctor and nurse from the Children's Emergency Department.

What are the side effects to be aware of?

- Your child may seem to be awake after receiving ketamine.
- Your child may move a little without obvious cause; this is normal.
- Your child's eyes may twitch; this is normal.
- Your child may report odd dreams on waking up, and may become a little agitated (less than 1 in 5 children experience this).
- 1 in 10 children develop a rash.
- 1 in 10 children are sick (vomit).
- 1 in 10 children will have watery eyes, or may drool.
- Very rarely the vocal cords can close. Extremely rarely this can mean a general anaesthetic needs to be given, with a breathing tube placed in their windpipe.

The hospital doctors are trained to recognise and deal with any complications your child may have.

**Health information and support is available at www.nhs.uk
or call 111 for non-emergency medical advice**

Visit www.leicestershospitals.nhs.uk for maps and information about visiting Leicester's Hospitals
To give feedback about this information sheet, contact InformationForPatients@uhl-tr.nhs.uk

How can I help my child?

Before your child is sedated it is good to reassure them by talking to them calmly and play with them quietly, for example, reading a book. You may stay with them during the procedure and staff may ask you to talk to your child during the procedure to help comfort them.

After the procedure, it will help for you to stay with your child so that they recognise a familiar face when they wake up. When they are awake, talk to your child and help keep them settled so they are relaxed as the medicine wears off.

How should I care for my child at home?

Most children wake up after a few minutes and will fully recover within 1 to 2 hours. Your child will be safe to go home once they are fully awake, can walk unaided, and can drink without being sick.

Sometimes after sedation children can be a bit confused, sleepy or clumsy and so you need to be watchful in caring for them for the next 24 hours. They should avoid activities such as riding a bike, sports and active play. Make sure they are supervised when bathing too.

Clear fluids (but not fizzy or sugary drinks) should be encouraged, but do not allow them to eat until at least 2 hours have passed at home without any problems. Give light meals only until the following day.

Your child may sleep as normal or be more tired than usual. They may also have more vivid dreams for 1 or 2 nights.

Contact details

If you have any concerns you can contact the NHS helpline on 111 for advice.

Giving us your feedback

We would love to get some feedback on your visit today. Use your smart phone to scan this QR code for quick access to our online feedback survey form.

Alternatively, you can access the feedback form from our website:

<https://www.leicestershospitals.nhs.uk/patients/thinking-of-choosing-us/patient-experience/>



اگر آپ کو یہ معلومات کسی اور زبان میں درکار ہیں، تو براہ کرم مندرجہ ذیل نمبر پر ٹیلی فون کریں۔
على هذه المعلومات بلغة أخرى، الرجاء الاتصال على رقم الهاتف الذي يظهر في الأسفل
જો તમને અન્ય ભાષામાં આ માહિતી જોઈતી હોય, તો નીચે આપેલ નંબર પર કૃપા કરી ટેલિફોન કરો

ਜੇ ਤੁਸੀਂ ਇਹ ਜਾਣਕਾਰੀ ਕਿਸੇ ਹੋਰ ਭਾਸ਼ਾ ਵਿਚ ਚਾਹੁੰਦੇ ਹੋ, ਤਾਂ ਕਿਰਪਾ ਕਰਕੇ ਹੇਠਾਂ ਦਿੱਤੇ ਗਏ ਨੰਬਰ 'ਤੇ ਟੈਲੀਫੋਨ ਕਰੋ।
Aby uzyskać informacje w innym języku, proszę zadzwonić pod podany niżej numer telefonu

If you would like this information in another language or format such as EasyRead or Braille, please telephone 0116 250 2959 or email equality@uhl-tr.nhs.uk